

How to Open a Home Care Agency in Colorado

A Step-by-Step Guide to Launching a Class B (Non-Medical) Home Care Agency

Personal care • Companion & homemaker services • Colorado

Built on the Colorado home care agency rules — 6 CCR 1011-1 Chapter 26 (CDPHE) — 2026 edition

General educational information, not legal advice. Verify every fee, figure, and phone number with the source before you rely on it.

Start Here — What This Guide Covers

This guide walks you from idea to open business as a Colorado Class B Home Care Agency — the non-medical license that lets you provide hands-on personal care, companion, and homemaker services in a client's home. Colorado runs a two-class system: a Class A agency provides skilled healthcare services (nursing, therapy), while a Class B agency provides personal care only. Class B is the lower-cost, lower-barrier door into Colorado's fast-growing senior-care market, and it is the focus of this guide.

Read Parts 1 through 4 before you spend real money — they tell you what a Class B agency may do and exactly how to get licensed through CDPHE, including Colorado's distinctive fingerprint-plus-CAPS-plus-DORA screening stack. Parts 5 and 6 are about revenue: where clients come from and how you get paid, including Colorado Medicaid's personal care benefit, Community First Choice, and Sandata Electronic Visit Verification. Parts 7 through 9 keep you compliant once you are open — including Colorado's strict labor rules and its mandatory-reporter law. The two worked examples show how it comes together in Denver and Colorado Springs, and the contacts, glossary, and references at the end are your quick-reference desk. Verify every fee, figure, and phone number before you rely on it — requirements change.

The one rule that defines your business

A Class B home care agency provides **personal care only**. It may not provide any skilled healthcare service — no nursing, wound care, injections, medication administration, or therapy. Your staff assist with activities of daily living and may help a client self-administer medication, but they never administer it. Cross that line and you are operating outside your license.

The Path at a Glance

Here is the whole journey in one place. Each step is explained in detail later in the guide.

1. **Decide your structure and register the business** with the Colorado Secretary of State (LLC or corporation), get an EIN, and file any trade name.
2. **Build your compliance foundation** — policies and procedures, forms, personnel and consumer-record systems, and a qualified manager/administrator.
3. **Secure liability coverage** of at least \$100,000 per occurrence and \$300,000 aggregate (the Class B minimum).
4. **Apply for your Class B license through COHFI** (the CDPHE online portal), and complete fingerprint, CAPS, and DORA screening before anyone provides care.
5. **Pass your CDPHE review and any on-site survey**, and receive your license (a provisional license is possible, but never before the criminal background check is complete).
6. **Enroll with Colorado Medicaid (HCPF) if you want Medicaid clients**, and set up Electronic Visit Verification through Sandata or a compliant Provider Choice System.
7. **Set up labor compliance** — Colorado minimum wage and daily overtime, and workers' compensation insurance (required at one or more employees).
8. **Build referral relationships and start serving clients**, following your plan-of-care, supervision, and mandatory-reporting duties.

Part 1 — What a Class B Home Care Agency Is (and Isn't)

Colorado licenses home care agencies under 6 CCR 1011-1 Chapter 26 and Chapter 2, adopted under C.R.S. Title 25, Article 27.5, and administered by the Colorado Department of Public Health and Environment (CDPHE), Health Facilities Division. A home care agency provides home care services to consumers in their place of residence. Colorado divides these agencies into two classes, and choosing the right one is the first decision you make.

Class A vs. Class B

Class A agencies provide skilled healthcare services — anything that by its complexity requires a licensed nurse or therapist, such as skilled nursing, wound care, injections, medication administration, and physical, occupational, or speech therapy. A Class A agency may also provide personal care. **Class B** agencies provide personal care only and may not provide any skilled healthcare service. Class B carries a lower liability-coverage minimum (\$100,000/\$300,000 versus \$500,000/\$3,000,000 for Class A) and does not require Medicare certification, which makes it the natural entry point for a new non-medical business.

What personal care includes

Personal care means assisting with, providing hands-on support for, or supervising personal care tasks to help a client with activities of daily living — bathing, dressing, grooming, mobility and transfers, eating, and toileting — together with home management such as light housekeeping, laundry, meal preparation, and shopping. Your personal care workers may remind a client to take medication and bring the container, but they do not measure doses, place medication in the client's mouth, or otherwise administer it. When a client needs a skilled service, you document the need and refer to a Class A agency, a home health agency, or the client's physician.

Good news for Colorado founders

There is no Certificate of Need to clear for a Class B agency — the gate is CDPHE licensure and passing survey, not proving unmet community need. If you build a compliant operation, you can get licensed.

Part 2 — Set Up the Business

Before you touch the license application, put the business itself in place.

- **Choose a legal structure and register with the Colorado Secretary of State.** Most founders form an LLC or corporation at sos.state.co.us. File any trade name (DBA) with the Secretary of State — Colorado files trade names at the state level, not the county.
- **Get an EIN from the IRS.** Free, online, and needed for banking, payroll, and enrollment.
- **Maintain a physical business office in Colorado.** Your agency must have a physical office capable of conducting day-to-day business; a purely virtual operation does not satisfy the rule.
- **Open business banking and set up bookkeeping and payroll.** Colorado's wage-and-hour rules (Part 7) make clean payroll essential from day one.
- **Line up liability coverage.** You will need proof of liability insurance or a surety bond of at least \$100,000 per occurrence and \$300,000 aggregate to license as Class B; get quotes early.

Part 3 — Get Licensed Through CDPHE

This is the heart of the process. You apply through the Colorado Health Facilities Interactive (COHFI) portal, complete Colorado's screening stack, and pass CDPHE review and any on-site survey.

The screening stack — do this before anyone provides care

Colorado requires three separate checks before any individual provides direct consumer care, and this is one of the most-missed requirements for new agencies:

- **Fingerprint-based criminal history check** through the Colorado Bureau of Investigation (CBI) and the FBI. The owner/applicant pays the costs, and no provisional license is issued before this check is complete.
- **CAPS check** — a query of the Colorado Adult Protective Services Data System under C.R.S. § 26-3.1-111, required before you employ anyone to provide direct consumer care.
- **DORA verification** — confirm through the Department of Regulatory Agencies that any license, registration, or certification a role requires is current and in good standing, and keep a copy of the inquiry in the personnel file.

The application

Through COHFI you submit your ownership and manager/administrator information, your policies and procedures, evidence of liability coverage, your declared geographic service area (the contiguous counties you will serve), and the applicable fee. CDPHE reviews the application, processes the background checks, and may schedule an on-site survey; processing commonly takes several months, so apply well before you plan to open. A provisional license may be issued under Chapter 2, but never before the criminal background check is complete.

About the fee

CDPHE sets home care agency license fees on its Facility Fees schedule (updated March 1, effective July 1), and the amount depends on factors like annual admissions and branches. Because it changes, this guide does not quote a number — confirm the current Class B fee on CDPHE's fee schedule before you budget. (Note: the widely-cited "\$870" figure is the separate registration fee for home care *placement* agencies, which is a different business.)

Your manager/administrator and service area

You must designate a manager/administrator who controls and supervises day-to-day operations and meets the Chapter 26 qualifications, and you must declare the contiguous Colorado counties you will serve, assuring adequate staffing and supervision throughout that area. Notify CDPHE within ten days of a change in owner, manager, or administrator, and thirty days before relocating.

Part 4 — Build Your Compliance Foundation

CDPHE licenses agencies that have their operational systems in place, not just a business plan. Before survey, you need:

- **A policy-and-procedure manual** built to Chapter 26 and Chapter 2, covering administration, consumer rights, personnel and screening, service delivery and supervision, health and safety, and quality — implemented, not just written.
- **A forms system** — intake, assessment, plan of care, service notes, supervisory-visit records, personnel and screening logs, and incident and complaint forms.
- **Personnel systems** — screening (fingerprint, CAPS, DORA), signed job descriptions, documented competency for each task, orientation and in-service training, and annual evaluations.
- **Consumer-record systems** — a plan of care established before services begin, service notes, and the 90-day onsite supervisory visit for personal care clients.
- **A quality management program** appropriate to your size that monitors supervision, complaints, incidents, and compliance.

PolicyReady.org's Colorado Class B package — the Policy & Procedure Manual, Forms Packet, Caregiver Handbook, and this guide — is built to give you exactly this foundation, ready to customize with your agency's details.

Part 5 — Where Clients Come From

A licensed agency with no referral pipeline has no revenue. Personal care clients reach you through a predictable set of channels, and your job is to build relationships in each:

- **Hospital discharge planners and case managers** who arrange in-home support for patients going home.
- **Skilled nursing and rehab facilities** discharging residents back to the community.
- **The aging network** — your Area Agency on Aging and Aging & Disability Resource Center, which field calls from families every day.
- **Medicaid case management agencies** that authorize and coordinate waiver and Community First Choice services.
- **Assisted living communities, hospices, elder-law attorneys, and geriatric care managers** who meet families at decision points.

Build these relationships lawfully — see Part 8 on the anti-kickback line. PolicyReady.org's *Referral System (Colorado Edition)* gives you a verified directory and metro-by-metro target lists to work from.

Part 6 — How You Get Paid

Colorado home care is funded through several channels. Most new agencies combine private pay with Colorado Medicaid.

Funding source	What it covers	How to access it
Private pay	Clients or families pay directly, per your schedule of charges	Your service agreement; strongest early revenue
Colorado Medicaid — Personal Care & Homemaker (HCBS/EBD)	Personal care and homemaker services for eligible waiver clients	Enroll with HCPF; work with case management agencies; EVV required

Funding source	What it covers	How to access it
Community First Choice (CFC)	State-plan personal care; requires a Class A or B license since 7/1/2025	Enroll with HCPF; hold your Class B license
CDASS / IHSS (consumer-directed)	Consumer-directed attendant support and in-home support options	Coordinate with HCPF and case management agencies
Veterans programs	In-home support for eligible veterans	VA (1-877-222-8387); coordinate with VA social work
Long-term care insurance	Private LTC policies that cover in-home personal care	Verify the client's policy terms and billing process

Colorado Medicaid is administered by the Department of Health Care Policy and Financing (HCPF), separately from your CDPHE license. To bill Medicaid you enroll as a provider through the HCPF provider enrollment portal, and — because of Community First Choice — you must hold your Class A or Class B license to provide personal care under CFC. Personal care and homemaker services are delivered under Health First Colorado, the Elderly, Blind and Disabled (EBD) and other HCBS waivers, CFC, and consumer-directed options (CDASS and IHSS).

Electronic Visit Verification (EVV)

Colorado requires Electronic Visit Verification for Medicaid personal care under 10 CCR 2505-10 § 8.001. Colorado uses a hybrid, open-choice model: you may use the State EVV Solution provided by Sandata at no cost — its mobile app, telephone option, and provider portal — or a Provider Choice System (your own vendor) configured to Colorado's rules and interfaced with the state aggregator. Since February 1, 2022, claims that require EVV must match a valid EVV record to be paid, so set this up before you bill. Colorado does not accept biometric, photo, or video data.

Part 7 — Colorado Labor Rules (Read This Carefully)

Colorado's wage-and-hour rules are stricter than federal law and stricter than most states, and home care agencies get tripped up here. Build these into your pay practices from day one.

- **Minimum wage.** Colorado's minimum wage is higher than the federal wage and adjusts every year for inflation (it is \$15.16 per hour for 2026). Several localities set higher rates — Denver, Boulder, Boulder County, and Edgewater — and the higher applicable rate governs. Verify the current figure for your location.
- **Daily overtime.** Under the COMPS Order, you pay overtime for hours worked over 40 in a week, over 12 in a day, or over 12 consecutive hours — whichever produces the higher pay. This daily-overtime rule catches agencies used to weekly-only overtime. (Medicaid-funded workers on 24-hour shifts are exempt from the 12-hour daily overtime.)
- **Meal and rest periods.** A 30-minute duty-free meal period when a shift exceeds 5 hours, and a paid 10-minute rest period for each 4 hours worked (with some flexibility for Medicaid home care).
- **Workers' compensation.** Colorado requires workers' compensation insurance for any employer with one or more employees — including part-time. Get coverage before your first hire and report injuries to your carrier within the required timeframe.
- **Pay travel time and keep records.** Compensable travel between clients counts as hours worked; give itemized pay statements and keep pay records at least three years.

Part 8 — Staying Compliant Once You're Open

The 90-day supervisory visit

For Medicaid personal care clients, you must conduct an onsite supervisory visit at least every 90 days — to resolve problems, validate the worker's skills, provide client-specific training, observe the client's condition, and assess satisfaction — with at least one assigned personal care worker present. Track these from each client's start date so none is missed; apply the same practice to private-pay clients as good practice.

Mandatory reporting — a Colorado duty you must train

Under C.R.S. § 18-6.5-108, home care providers are mandatory reporters. If you observe or reasonably suspect mistreatment — physical or sexual abuse, caretaker neglect, or exploitation — of an at-risk elder (age 70 or older) or an at-risk adult with an intellectual and developmental disability, you must report it to law enforcement within 24 hours, in addition to reporting internally to your manager. A willful failure to report is a criminal offense. Train every staff member on this duty and keep a law-enforcement contact list for the counties you serve.

The anti-kickback line

You may build referral relationships, but you may not pay for referrals. The federal Anti-Kickback Statute and Colorado law prohibit offering, paying, soliciting, or receiving anything of value to induce referrals of Medicaid-funded clients. Keep any arrangement with a referral source documented and for fair value for bona fide services — never tied to the volume of referrals.

Renewals and notifications

Renew your Class B license annually through COHFI before it expires. Notify CDPHE within 10 days of a change in owner, manager, or administrator, and 30 days before relocating. If CDPHE cites a deficiency, submit a plan of correction; the department may impose restrictions and civil fines up to \$10,000 per year.

Part 9 — Common Mistakes to Avoid

- **Letting a worker start before screening is complete.** Fingerprint, CAPS, and DORA checks must be done before any direct consumer care — no exceptions.
- **Drifting across the Class B scope line.** The moment staff administer medication or perform a skilled task, you are operating outside your license.
- **Treating overtime as weekly-only.** Colorado's 12-hour daily overtime rule is easy to miss and expensive to get wrong.
- **Skipping workers' comp because you're "small."** Colorado requires it at one employee.
- **Missing the 90-day supervisory visit.** Track due dates from each client's start; a missed visit is a compliance and billing problem.
- **Billing before EVV is set up.** Claims requiring EVV won't pay without a matching record.
- **Not training the mandatory-reporter duty.** Every staff member needs to know the 24-hour, report-to-law-enforcement rule.
- **Inventing dollar figures in your budget.** Verify the current license fee, wage, and rate figures from the source — they change.

Part 10 — Growing Your Agency Once You're Established

Once you are licensed and serving clients steadily, Colorado gives you several ways to grow — each with its own compliance steps.

- **Expand your service area.** You can add contiguous counties to your declared geographic service area, but only if you can assure adequate staffing and supervision — including the 90-day supervisory visit — across the larger area. Update your declaration with CDPHE before you accept clients in the new counties.
- **Open a branch office.** A branch extends your reach within your service area. Notify CDPHE in advance, offer only the same services as the parent, and ensure the parent supervises the branch daily. A branch is not a way to offer new service types — it mirrors the parent.
- **Add consumer-directed options.** As you build Medicaid volume, understand how CDASS and IHSS work so you can serve clients who direct their own care, and coordinate with the case management agencies that administer them.
- **Deepen your referral relationships.** Your earliest hospital, skilled-nursing, and aging-network contacts become your steadiest source of clients once they trust your reliability and documentation. Consistency — showing up, communicating, and never crossing scope — is what earns repeat referrals.
- **Consider the move to Class A later.** If you eventually want to provide skilled services (nursing, therapy), that is a Class A license — a bigger step with a higher liability minimum (\$500,000/\$3,000,000), Medicare certification, and clinical supervision requirements. Many founders start Class B, build a strong operation, and evaluate Class A once they have scale and management depth.

Whatever path you choose, the compliance disciplines you build now — screening, supervision, documentation, mandatory reporting, and clean payroll — are exactly what let you grow without losing control of quality or your license.

Your First 90 Days

A realistic sequence once you commit:

1. **Weeks 1-2:** Register the business with the Secretary of State, get your EIN, open banking, and get liability-coverage quotes.
2. **Weeks 3-6:** Stand up your compliance foundation — policies, forms, personnel and consumer-record systems — and designate your manager/administrator.
3. **Weeks 5-8:** Start the COHFI application, begin fingerprint/CAPS/DORA screening, and secure liability coverage.
4. **Weeks 8-12:** Respond to CDPHE, prepare for any on-site survey, set up payroll and workers' comp, and begin referral outreach so clients are waiting when your license issues.
5. **After licensure:** Enroll with HCPF and set up EVV if serving Medicaid clients, then onboard your first clients with plans of care and your 90-day supervisory schedule in place.

Worked Example 1 — Opening in Metro Denver

Imagine you are opening a Class B agency serving Denver and its contiguous counties. Here is how the pieces connect in this market.

- **Your license and service area:** You license through CDPHE and declare Denver plus contiguous counties (for example Adams, Arapahoe, Jefferson, Douglas) as your service area — confirming you can staff and supervise the whole area.
- **Your aging-network anchor:** The DRCOG Area Agency on Aging (1001 17th St., Suite 700, Denver 80202; 303-480-6700) is the region's hub and Aging & Disability Resource Center — introduce yourself and understand its referral pathways.
- **Hospital discharge relationships:** Major systems whose case managers arrange home support include UCHHealth University of Colorado Hospital (Aurora; 720-848-0000), Denver Health (303-436-6000), Intermountain Health Saint Joseph Hospital (303-812-2000), and HCA HealthONE Rose (303-320-2121).
- **Getting paid:** You serve private-pay clients from day one and, once enrolled with HCPF and set up on Sandata EVV, add Medicaid personal care clients referred by case management agencies.

Worked Example 2 — Opening in Colorado Springs

Now imagine opening in the Pikes Peak region.

- **Your license and service area:** You license through CDPHE and declare El Paso plus contiguous counties (for example Teller, Pueblo, Douglas) as your service area, assuring staffing and supervision throughout.
- **Your aging-network anchor:** The Pikes Peak Area Agency on Aging (15 S. 7th St., Colorado Springs 80905; 719-471-7080), part of the Pikes Peak Area Council of Governments, serves El Paso, Park, and Teller counties and is your regional hub.
- **Hospital discharge relationships:** Case managers who arrange home support work at UCHHealth Memorial Hospital Central (719-365-5000), CommonSpirit Penrose Hospital (719-776-5000), CommonSpirit St. Francis Hospital (719-571-5000), and UCHHealth Grandview Hospital (719-365-3300).
- **Getting paid:** As in Denver, combine private pay with Colorado Medicaid once you are enrolled with HCPF and live on EVV.

Finding Your Own County's Contacts

Wherever you operate in Colorado, the same local contacts exist under different names. To build your own list:

1. Call the statewide aging line or 2-1-1, or use the national Eldercare Locator (1-800-677-1116), to reach your county's Area Agency on Aging and Aging & Disability Resource Center.
2. Identify the hospitals and skilled-nursing facilities in your service area and ask each for the case-management or discharge-planning department.
3. Find your county Department of Human/Social Services (for adult protective services) and your county law enforcement non-emergency line (for your mandatory-reporting duty).
4. Enroll with HCPF and set up Sandata EVV; connect with the case management agencies that authorize waiver and CFC services in your area.
5. Introduce yourself to assisted living communities, hospices, elder-law attorneys, and geriatric care managers in your area.

Colorado — Who to Call

Verified Colorado and national contacts for building your agency. Confirm each before you rely on it — government numbers change.

For...	Who to contact
CDPHE Health Facilities Division (licensing)	Apply/renew via the COHFI portal · cdphe.colorado.gov
Colorado Medicaid — HCPF Provider Services	1-844-235-2755 · hcpf.colorado.gov
Sandata — Colorado EVV support	1-855-871-8780 · CoCustomerCare@sandata.com
CDLE — Division of Labor Standards & Statistics (wage/hour)	303-318-8441 · cdle.colorado.gov/dlss
Colorado Secretary of State (business/DBA)	303-894-2200 · sos.state.co.us
DORA (license verification)	303-894-7855 · dora.colorado.gov
DRCOG Area Agency on Aging (Denver region)	303-480-6700 · drcog.org
Pikes Peak Area Agency on Aging (Colorado Springs)	719-471-7080 · ppacg.org
Adult Protective Services	Report to your county Dept. of Human/Social Services
Mandatory report of at-risk-adult mistreatment	Call local law enforcement within 24 hours; 911 if in immediate danger
Eldercare Locator (find local aging services)	1-800-677-1116 · eldercare.acl.gov
Colorado 2-1-1 (health & human services)	Dial 2-1-1 · 211colorado.org

Glossary

For...	Who to contact
Class B home care agency	A Colorado agency licensed to provide personal care only — no skilled healthcare service.
Personal care worker (PCW)	An agency employee qualified by training or verified experience to provide personal care.
CAPS check	A check of the Colorado Adult Protective Services Data System, required before hiring direct-care staff.
DORA	The Department of Regulatory Agencies; you verify credentials in good standing through DORA.
COHFI	Colorado Health Facilities Interactive — CDPHE's online licensing portal.
HCPF	The Department of Health Care Policy and Financing, which runs Colorado Medicaid.
Community First Choice (CFC)	A Medicaid state-plan option; personal care providers must hold a Class A/B license.
EVV	Electronic Visit Verification — electronic capture of Medicaid visit data (via Sandata or a Provider Choice System).
COMPS Order	Colorado's Overtime and Minimum Pay Standards Order — wage, overtime, and break rules.

For...	Who to contact
Service note	A signed, dated, timed note documenting a non-medical service (the Class B documentation term).
At-risk elder	Any person 70 or older, for purposes of Colorado's mandatory-reporting law.
Plan of care	The written plan of the personal care tasks and frequency for a client, established before services begin.

Core Rules & References

- **6 CCR 1011-1 Chapter 26** — Home Care Agencies (Colorado licensing rules, including Class A/B).
- **6 CCR 1011-1 Chapter 2** — General Licensure Standards (screening, provisional license, plans of correction).
- **C.R.S. Title 25, Article 27.5** — the home care agency statute.
- **C.R.S. § 26-3.1-111** — CAPS (Adult Protective Services Data System) check.
- **C.R.S. § 18-6.5-108** — mandatory reporting of at-risk-adult mistreatment.
- **10 CCR 2505-10 § 8.500** — Medicaid Personal Care benefit (90-day supervisory visit, tasks).
- **10 CCR 2505-10 § 8.001** — Electronic Visit Verification.
- **Colorado COMPS Order (7 CCR 1103-1) & C.R.S. Title 8** — wage/overtime and workers' compensation.

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